



TDMHSAS FY 2024 Joint Annual Report

Leading the Nation

Tennessee Department of Mental Health & Substance Abuse Services

Joint Annual Report SFY24



Executive Summary

The Tennessee Department of Mental Health and Substance Abuse Services (TDMHSAS or Department) functions as Tennessee's mental health, substance use disorders (SUDs), and opioid treatment authority. The Joint Annual Report (JAR) allows TDMHSAS, jointly with the Statewide Planning and Policy Council membership, to report accomplishments and challenges annually to the Governor and State Legislature. During each fiscal year, TDMHSAS conducts a Needs Assessment that focuses on the population of Tennessee to ascertain unmet service needs and delivery system gaps. In the subsequent year, TDMHSAS develops funding and program targets that address the service needs identified by the assessment.

In FY 2024, TDMHSAS continued to engage in collaborations to improve service outcomes while containing costs; maintaining and improving community mental health and substance abuse services; providing effective education and prevention services; decreasing prescription drug abuse; and promoting wellness and recovery for the citizens of Tennessee.

One ongoing challenge for TDMHSAS is maintaining a high-quality continuum of services while facing increased demands. As a response to the challenge, TDMHSAS leverages state, federal, and other non-state resources to meet unmet needs. In FY 2024, with noteworthy support from Governor Bill Lee and the Tennessee General Assembly (TNGA), the Department continued to work deliberately to increase suicide prevention and awareness while reducing the stigma associated with behavioral health, increasing substance abuse funding, and providing effective continuity of care for individuals living with mental health, substance use, and co-occurring disorders (CODs). In all, the FY 2024 budget of \$731,661,600, including \$497,012,000 in state appropriations, allowed the Department to continue to work toward its goal of ensuring that Tennesseans needing mental health or SUD treatment can connect with services regardless of their ability to pay.

Additional FY 2024 successes for TDMHSAS include: addressing behavioral healthcare workforce issues through the Tennessee Behavioral Health Pathways (TBHP) Scholarship, which provided tuition stipends to Tennessee residents pursuing a graduate degree in a behavioral health related field, and the Public Behavioral Health Workforce Sign-On and Retention Bonus Program, which provided bonuses for over 3,000 behavioral health workers in Tennessee in an effort to bolster and retain the behavioral healthcare workforce in Tennessee; the establishment of a children's crisis stabilization unit (CSU) serving individuals ages 4-17; ; and the launch of the Residential Re-Entry Housing Program (RRHP) which provides long-term quality housing with supportive residential services for Tennesseans experiencing Severe and Persistent Mental Illness (SPMI) who re-enter the community from custody of the Tennessee Department of Correction (TDOC), incarceration in county jails, or are justice-involved.

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Purpose, Scope, and Activities of TDMHSAS

It is the mission of [TDMHSAS](#) to create collaborative pathways to resiliency, recovery, and independence for Tennesseans living with mental illness and SUDs, and the Department's vision is a state of resiliency, recovery, and independence in which Tennesseans living with mental illness and SUDs thrive.

TDMHSAS provides services to individuals and families in Tennessee struggling with mental health and substance abuse issues, providers, legislators, other state agencies, and consumer/advocacy groups. Those services include education; prevention; early intervention; treatment; licensing oversight for community organizations providing behavioral health services; and outpatient and inpatient care. The Department operates four (4) Regional Mental Health Institutes (RMHIs) and provides research and policy development regarding a wide array of behavioral health issues.

The Department is responsible for system planning; setting policy and quality standards; licensing personal support services agencies, mental health, and substance use facilities; system monitoring and evaluation; disseminating public information; and, advocating for persons of all ages who live with serious mental illness (SMI), serious emotional disturbance (SED), SUD, and/or COD. Through the operation of four (4) fully accredited RMHIs, TDMHSAS also provides inpatient psychiatric services for adults, including acute, sub-acute, and forensic patients.

TDMHSAS is comprised of the following Department Offices and Divisions: Office of the Commissioner; Office of Communications; Office of Faith-Based Initiatives; Division of Fiscal Services; Office of Forensic and Juvenile Court Services; Office of Human Resources; Office of Strategic Initiatives; Division of Administrative and Regulatory Services; Division of Clinical Leadership; Division of General Counsel; Division of Hospital Services; Division of Mental Health Services; Division of Children and Youth Mental Health Services; Division of Planning, Policy & Legislation; and the Division of Substance Abuse Services. Through the Department Offices and Divisions, TDMHSAS provides a quality spectrum of behavioral health services across the lifespan. Collaborative efforts, both public and private, involve partnerships with mental health, substance abuse, criminal justice, veterans, and child/family organizations, etc. The result is the creation of a cross-systems approach that promotes the most effective outcome of care.

TDMHSAS is dedicated to implementing and executing Governor Bill Lee's vision of a state government that is customer-focused, efficient, and effective. The goals of this vision, commonly referred to as Customer Focused Government (CFG), include efficient and effective management of the RMHIs; maintaining and improving community mental health and substance abuse services; and providing effective education and prevention services.

Statewide and Regional Planning and Policy Council System

Title 33 of the Tennessee Code Annotated (T.C.A) requires a structured planning process with council participation. TDMHSAS administers seven (7) Regional Planning and Policy Councils from which regional mental health and substance abuse needs and information are channeled to the Statewide Planning and Policy Council and to the Department. Current or former service recipients and/or members of service recipient families are a vital part of the membership of each council. All levels of this unified planning and policy council system are advisory. Each Council meets quarterly.

Statewide Planning and Policy Council

The purpose of the TDMHSAS Statewide Planning and Policy Council is to assist in planning a comprehensive array of high-quality prevention, early intervention, treatment, and habilitation services and supports, and to advise the Department on policy, budget requests, and developing and evaluating services and supports (T.C.A. §33-1-401).

The TDMHSAS Statewide Planning and Policy Council is made up of mental health and substance abuse services consumers, family members, providers, advocates, and other stakeholders in Tennessee. It is a large, active, independent body. In FY 2024, ten (10) council members were appointed or reappointed. The Statewide Planning and Policy Council achieved a quorum at each one of its quarterly meetings and actively sought to advise the Department concerning the needs of the communities served by its membership. In FY 2024, an average of 74 individuals participated in each of the quarterly meetings. Council committees (Adult Committee, Children's Committee, Executive Committee, Legislative Committee, and Planning and Budget Committee) met a total of 15 times during the fiscal year.

Regional Planning and Policy Councils

The purpose of the Regional Planning and Policy Councils is to advise the TDMHSAS Statewide Planning and Policy Council on the Three-Year Plan, including the desirable array of prevention, early intervention, and treatment services and supports for service recipients and their families, and such other matters as the Commissioner or the TDMHSAS Statewide Planning and Policy Council may request and provide information and advice to the Department on policy, formulation of budget requests, and development and evaluation of services and supports (T.C.A §33-2-202).

The Regional Planning and Policy Councils provide citizen participation in policy planning, and are representative of service recipients and their families, advocates for children, adults, the elderly, service providers, agencies, and other affected persons and organizations (T.C.A. §33-2-203). To achieve inter-related

communication and work among and between councils, the Chairs and Vice-Chairs of each Regional Council serve on the TDMHSAS Statewide Planning and Policy Council.

A Needs Assessment is conducted annually by the Regional Planning and Policy Councils to assist the Department with planning for resource allocation. Prioritized needs are shared with TDMHSAS staff to inform the development of strategies for the Three-Year Plan and report progress annually. In addition to the Needs Assessment, the Regional Councils also review and provide input on both the federal Mental Health and Substance Abuse Block Grant (MHBG and SABG) plans and funding, legislative proposals for review by the Commissioner and possible consideration by the Governor, and other Departmental reports and initiatives.

Regional Planning and Policy Councils are kept informed about Department activities through the monthly Executive Staff Report, in-person reporting at each quarterly Regional Planning and Policy Council meeting, and ongoing interaction via email and telephone provided by the Office of Planning. In addition to the information dissemination that takes place between the Department and the Councils, technical assistance regarding resources and the use of data in the Needs Assessment process were provided to representatives from the Regional and Statewide Councils in the first two months of calendar year 2024. The Office of Planning produces a Grants Finder Resource that is distributed via email and published on the Department's website monthly for use by the Councils for finding possible sources of funding. The Planning and Policy Council system is unique to Tennessee and serves to bring grass roots participation from all regions of the state.

Annual Needs Assessment Process

Identifying the most relevant behavioral health needs of Tennesseans is essential to the activities of the Department. TDMHSAS ensures that the most relevant needs are prioritized by asking the Statewide and Regional Planning and Policy Councils to complete an annual Needs Assessment. Each Spring, the seven (7) Regional Planning and Policy Councils as well as the Statewide Planning and Policy Council's Committees (Adult, Children's, and Consumer Advisory Board (CAB)) work independently to identify and prioritize up to three (3) mental health and three (3) substance abuse needs. Each identified need is supported by data and is submitted to the Department. Information from each Statewide Committee and Regional Planning and Policy Council is gathered, and a Needs Assessment summary is compiled. This summary is then shared with TDMHSAS leadership and assists in the development of the Department's Three-Year Plan.

The Needs Assessment process creates a data-informed method for the Statewide and Regional Planning and Policy Councils to influence the design of the mental health and substance use service delivery system by identifying each Region's needs to enable targeting of state resources to meet identified needs more effectively and efficiently. Participants in the Needs Assessment process include Statewide and Regional Planning and Policy Councils, consumers, family members, caregivers, advocates, service providers, and TDMHSAS staff. Considerations include the Governor's and Commissioner's priorities, state and federal law and regulations, SAMHSA strategic initiatives, data from statewide Needs Assessments, and funding

availability. During FY 2024, the Office of Research continued to maintain a variety of interactive dashboards on the Fast Facts Portal that were first developed in FY 2018. This portal, which provides budget information as well as an overview of key statistics concerning TDMHSAS-funded mental health and substance abuse services, is particularly helpful to the Statewide and Regional Planning and Policy Councils in identifying needs throughout the state. The Fast Facts Portal is available on the TDMHSAS website at [TN.gov/behavioral-health/research/fast-facts](https://tn.gov/behavioral-health/research/fast-facts).

Accomplishments, Activities, and Initiatives

Division of Mental Health Services

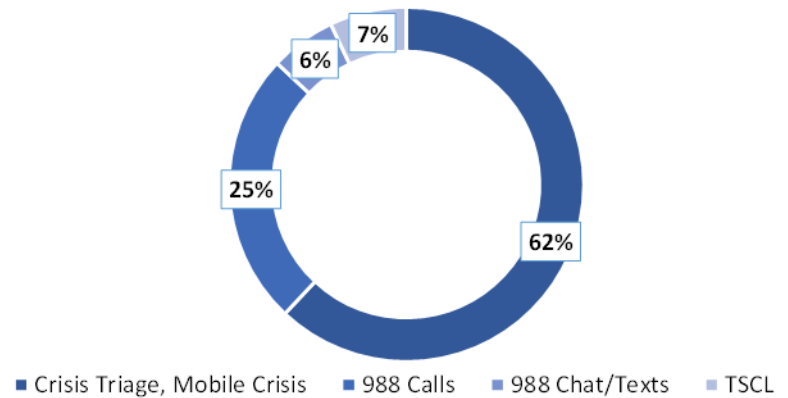
The Division of Mental Health Services (DMHS) administers and supports a diverse array of services and supports for individuals of all ages living with mental illness, COD, and SED. The DMHS creates, expands, and oversees community-based programs and community support services including affordable housing programs; homelessness prevention services; a full continuum of 24-hour crisis services; wellness and recovery services; peer recovery services; supported employment services; suicide prevention services; geriatric services/PASRR services; and disaster emergency services. All program services are delivered via a network of mental health providers from grant contracts to mental health services agencies in the community.

Community mental health providers (CMHPs) were grateful for the provider rate increases added to their current grant contracts in FY 2024. There was more than \$16 million in state recurring funds added to CMHPs for this purpose over the last two state fiscal years. Workforce development continued to be a focus in FY 2024. The Mental Health Services and Bonuses to Medical Practitioners Program was created by a non-recurring \$50 million appropriation (SB7089/HB7070, Section 8) resulting from the special session in FY 2024 regarding public safety. The program provides grants to licensed community mental health agencies that contracted with TDMHSAS in FY 2022-2023 to provide mental health services. Program funds are to be used by the agencies to provide mental health services and bonuses to medical practitioners as defined by T.C.A § 63-1-201. Over 700 medical practitioners received bonuses from these funds in FY 2024. These funds will continue until June 30, 2026.

988 Suicide and Crisis Lifeline and Statewide Crisis Line

The 988 Suicide and Crisis Lifeline celebrated its second anniversary on July 16, 2024. There are now eight (8) 988 Suicide and Crisis Lifeline providers in Tennessee. All 95 counties in Tennessee are equipped with both a primary and backup 988 provider, ensuring that 988 calls are answered around the clock, every day of the year. Chat and text services are also offered 24/7/365 by Tennessee-based providers as we continue to build capacity over the grant period across the state.

FY 2024
Tennessee's Crisis Continuum: "Front Doors"



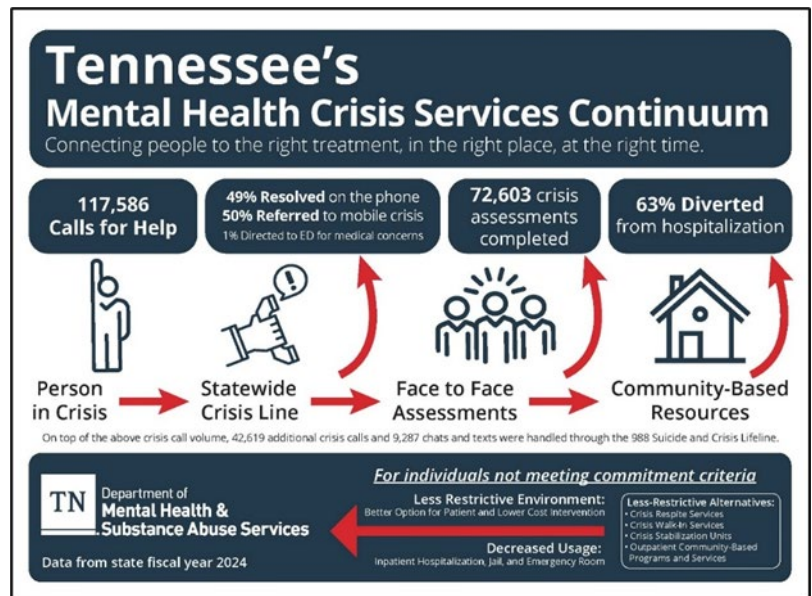
Through the agency level community-based provider Crisis Triage lines, the Tennessee Statewide Crisis Line (TSCL) and 988 Suicide and Crisis Lifeline, providers handled 160,205 total calls and 9,287 chat/texts for a total of 169,492 contacts with the Tennessee Crisis Continuum in FY 2024. Of the total crisis calls handled, 42,619 were through the 988 Suicide and Crisis Lifeline/988 implementation grant, and 117,586 calls were through the TSCL and Crisis Triage Lines through mobile crisis providers. Of the total calls handled through the Crisis Triage/Mobile Crisis providers and the TSCL, 79% of calls were regarding adults in crisis and 21% were for children and youth. Further, 50% were resolved by phone without the need for a face-to-face (F2F) crisis assessment. Types of resolution include referral for outpatient services, coordination of care with existing providers, and de-escalation.

During FY 2024, TDMHSAS entered the final year of the SAMHSA 988 Cooperative Agreement and was also awarded a Supplement to the SAMHSA 988 Cooperative Agreement, which increased capacity for chat/text, start-up funds for marketing, and allowed our evaluation provider to research staff training and workforce retention needs. In FY 2024, TDMHSAS was awarded SAMHSA's 988 State Improvement for Local 988 Capacity Grant. This multi-year funding allows for sustainment of staffing from the 988 Cooperative Agreement and Supplemental closeout. The 988 Improvement funding will also allow Tennessee's 988 centers to continue to increase staff capacity with calls and chat/text. Additionally, TDMHSAS is allocating the MHBG Crisis Set Aside funding to support the 988 Suicide and Crisis Lifeline infrastructure to increase capacity/staffing to allow for an increased in-state crisis call answer rate.

Crisis Continuum

Mobile Crisis Services are available to all Tennesseans, regardless of payor. However, most individuals receiving F2F crisis assessments are insured through TennCare (42%) or are uninsured (36%). Other types of payors identified include commercial insurance, Medicare, and Veterans Affairs (VA) benefits. Most assessments (43%) are completed in the emergency department (ED), while 26% are taking place in the Crisis Walk-in Centers (WICs).

During FY 2024, of the 72,603 total F2F crisis assessments, 82% were completed for adults and 18% were completed for children and youth. Of the total F2F crisis assessments, 63% were diverted from inpatient treatment and referred to alternative care (23-hour Observation, Crisis Stabilization Unit (CSU), Crisis Respite, or outpatient services). During FY 2024, there were nine (9) Adult Crisis Walk in Centers (WICs) and Crisis Stabilization Units (CSUs) across Tennessee (134 beds), providing crisis assessment, 23-hour observation, crisis stabilization and respite services as well as a CSU site dedicated to serving children ages 4-17 (12 beds).



Crisis WICs provide 23-hour direct service resources for individuals who would benefit from short-term stabilization in a supervised treatment environment and whose medical needs can be managed safely. In FY 2024, the average length of stay (LOS) in 23-hour observation at a Crisis WIC was 17.2 hours. CSU services are licensed by the state to offer 24/7/365 intensive, short-term stabilization, up to 120 hours, to assure that adequate arrangements are in place to allow the safe discharge of the service recipient. This service is for individuals who do not meet the criteria for other treatment resources, other less restrictive treatment resources are not available, or the service recipient is agreeable to receive services voluntarily at the CSU and meet admission criteria. In FY 2024, there were 6,408 admissions to the CSUs with an average LOS of three (3) days for adults and four (4) days for children.

Crisis Respite services are non-hospital, facility-based services, focused on short-term stabilization, up to 72 hours, offering services 24/7/365 in support of behavioral health treatment as determined by the crisis services provider. In FY 2024, there were 3,906 individuals who received respite care with an average LOS of 23 hours.

Suicide Prevention

During FY 2024, TDMHSAS and contracted partners collaborated to address suicide prevention utilizing a public health approach. This was achieved through a comprehensive range of initiatives, including gatekeeper training, awareness campaigns, and community outreach across all 95 counties.

Statewide suicide prevention training programs are offered to students, educators, pediatricians, and the public in multiple formats including in-person, virtual, and online video streaming, to ensure broad accessibility. In FY 2024, over 90,000 individuals were trained in youth suicide prevention, risk identification,

and awareness across the state. Specifically, gatekeeper trainings reached 75,344 educators, 156 pediatric primary care providers, and 18,148 adults statewide. Additionally, more than 47,000 individuals were screened for suicide risk and connected to appropriate services. Screening and follow-up services were provided through the Tennessee Lives Count-Connect 2 federal grant, Mental Health America's online screening tools, and additional statewide screening programs.

Behavioral Health Safety Net (BHSN) and Older Adults

The BHSN is a state-funded program providing essential outpatient mental health services to uninsured Tennesseans ages 18 and older and uninsured/underinsured Tennessee children ages 3 to 17 who meet program eligibility criteria through a network of participating CMHPs. Essential services offered through the BHSN include assessment and evaluation, therapeutic interventions, case management, psychiatric medication management, laboratory tests related to medication management, pharmacy assistance and coordination, and transportation to BHSN services.

In FY 2024, BHSN for Adults served 34,325 unique individuals ages 18 and older. More than 630,000 units of outpatient mental health services were provided, with the most utilized services being psychiatric medication management, case management, and individual therapy. In addition, 59% of BHSN for Adult enrollees received a telehealth service and 15% of all BHSN for Adult services were provided via telehealth.

In FY 2024, BHSN for Children served 1,855 unique children ages 3-17. More than 14,000 units of service were provided, with the most utilized services being case management, individual therapy, and assessment/evaluation services. Of those children served, 27% enrolled in TennCare/Cover Kids and 47% received at least one telehealth service with 19% of all BHSN for Children services provided via telehealth. In March 2024, TDMHSAS launched a statewide media campaign for BHSN for Children that included paid advertisements on television and on social media.

With more than 140 physical BHSN sites across the state and the successful utilization of telehealth, the statewide BHSN of TN Provider Network of 15 CMHPs can serve eligible adult Tennesseans no matter the county where they reside. In March 2023 in collaboration with the TDOH, the co-located Mental Health Services at County Health Departments Pilot launched at the Jackson County Health Department (JCHD) with Volunteer Behavioral Health Care System (VBHCS) as the contracted mental health provider. Between July 2023 and June 2024, 85 referrals were made to VBHCS from JCHD, with 46 of those individuals completing an intake to begin mental health services. One of the opportunities realized from the pilot was the need for an on-site therapist. As a result, funding was reallocated to include an on-site therapist at least one day a week in FY 25.

In FY 2024, the Older Adults Program (OAP) contracted with six (6) CMHPs. Combined, these six (6) providers enabled OAP to serve 52 counties in Tennessee. The OAP is designed to provide behavioral health care

coordination to individuals over the age of 50 who are not eligible for these services through TennCare or any other funding source. Care management services may include collaboration with other health care providers, community outreach providing older adult-related behavioral health education on healthy aging best practices, and older adult mental health concerns in the local community, depression screening assessments, in-home therapy via telehealth or face-to-face visits for older adults with limited mobility, person-centered advocacy, and referral/linkage to community resources like respite care and other supportive health services for older adults, their families, and caregivers. Older adult care coordination was provided to 478 unduplicated individuals in FY 2024.

In FY 2024, the Office of BHSN and Older Adults continued to fulfill the Department's Interagency Agreement with the Division of TennCare. This partnership executes the federal mandate for the state's mental health authority to finalize all Level II PASRR determinations for residents/applicants of Medicaid-certified nursing facilities in Tennessee. These reviews ensure residents with SMI diagnoses and related conditions are appropriately placed in the most independent setting and receive specialized mental health services as part of their plan of care. In FY 2024, the Office of BHSN and Older Adults reviewed and finalized 6,202 Level II PASRR evaluations.

Consumer Affairs and Peer Recovery Services

TDMHSAS operates a Helpline via phone, text, and email staffed by Peer Advocates who help callers in need of mental health and substance abuse resources in their area, questions about insurance, housing options, the Certified Peer Recovery Specialist (CPRS) program, or need help filing a complaint. In FY 2024, the advocates addressed 3,856 cases, which were received by phone, email, licensure staff, state legislators, and the Governor's Office. The Helpline Peer Advocates provide resources, referrals, ombudsman services, and assistance with complaints.

In FY 2024, TDMHSAS funded 45 Peer Support Centers in Tennessee where CPRS provided peer support and recovery assistance to 6,290 individuals who live with SMI. The Peer Recovery Call Center, operated by CPRS in East Tennessee at the Mental Health Association of East Tennessee (MHAET), received or made 5,247 calls helping Tennesseans seeking mental health resources and support. The Office of Consumer Affairs and Peer Recovery Services also continued its partnership with the Tennessee Department of Correction (TDOC), resulting in six (6) 40-hour trainings for 57 qualified inmates to become CPRS and provide peer support to their fellow inmates during their incarceration. In addition, workforce development opportunities were also provided to 693 peers through the standard CPRS training. There were 1,233 Certified Peer Recovery Specialists in the state of Tennessee in FY 2024 providing peer support services throughout the behavioral health system.

Lastly, the Department continued its partnership with the National Alliance on Mental Illness (NAMI) Tennessee, to expand Crisis Intervention Team (CIT) programs throughout the state. TDMHSAS funds CIT

statewide with MHBG funds to help maintain the statewide CIT in TN Task Force, expand CIT statewide, implement effective community strategies to better serve individuals in psychiatric crisis, safely reduce the prevalence of individuals with behavioral health needs in local jails, reduce costs related to prosecution and incarceration, and improve public safety. In FY 2024, 695 law enforcement officers and other first responders were trained in CIT.

Housing and Homeless Services

TDMHSAS continues to progress its charge to sustain and enhance the delivery of housing and homeless services, both qualitatively and quantitatively.

The flagship program of TDMHSAS' Housing and Homeless Services, the Creating Homes Initiative (CHI), was devised by current TDMHSAS Commissioner Marie Williams in the year 2000 to increase the safe, affordable, quality, permanent housing options for those Tennesseans experiencing mental illness or CODs. CHI implements an effective model of fostering assertive and strategic partnerships to leverage resources to achieve its goal. This effort is led by community-based Regional Housing Facilitators (RHF) who are "hands-on" community-based collaborators to help identify needs, lend expertise, share knowledge, and support grant writing efforts. In total, the Regional Housing Facilitators (RHF) led collaborative efforts and fostered effective and productive partnerships with various state, local, and federal entities to leverage over \$98 million to create over 1,361 affordable housing opportunities in FY 2024 bringing program lifetime totals to \$1.3 billion leveraged and more than 36,000 affordable housing opportunities created across Tennessee.

Intensive Long-term Support (ILS) is a program that implements the Permanent Supportive Housing model, which follows evidence-based practices to provide quality, affordable housing with comprehensive wrap-around support services to individuals discharging from the state's RMHIs. In FY 2024, a new ILS facility was completed and opened in Jackson, TN, serving discharges from Western Mental Health Institute (WMHI) and adding twenty (20) new beds to the program, all of which are now fully occupied.



Above: New 12-bed capacity home in Jackson, TN funded by the Creating Homes Initiative 2.0

Supported Employment

Individual Placement and Support (IPS) supported employment is an evidence-based program designed to help individuals living with behavioral health conditions return to and maintain jobs of their choosing. The program is delivered through a partnership between TDMHSAS and the Tennessee Department of Human Services' Division of Vocational Rehabilitation (TDHS-VR). Supported Employment programs help individuals with obtaining employment, and once employed, provide supports to assist with maintaining this employment. IPS is offered in 49 counties in Tennessee, 39 of which are rural, with the goal of providing services to all 95 counties. IPS services are provided in all TDHS-VR and TDMHSAS regions across the state. In FY 2024, 1,611 job seekers were supported by IPS teams across the state and 47% of those found employment. Of those who began working, 46% were employed for 90 days or more. This data exceeds the national average for IPS teams which currently is at a 42.5% competitive employment rate.

Peer Wellness

The Peer Wellness Initiative (PWI) is a peer-led health and wellness program offered in 27 counties, 17 of which are rural. Peer Wellness Coaches (PWCs) facilitate evidence-based workshops and provide one-on-one peer wellness coaching to help individuals improve their overall health and well-being. Workshop topics vary from chronic disease self-management programs, tobacco cessation, nutrition, increasing physical activity, and improving self-management behaviors.

In FY 2024, two (2) additional PWCs were added, increasing the total number of coaches to nine (9) PWCs and one Statewide PWI Director. In FY 2024, PWCs worked with 1,381 individuals to improve their health and wellness with 98% of individuals surveyed reporting an increase in self-management behaviors. In addition to workshops and one-on-one coaching, PWCs also facilitated 1,681 health and wellness activities.

Division of Children and Youth Mental Health Services

The Division of Children and Youth Mental Health Services (DCYMHS) continues to see growth in the number of programs offered by the Division as well as increases in funding. The continued increase in funding demonstrates the ongoing commitment of Governor Bill Lee and the TNGA to the mental health and well-being of the children, youth, young adults, and families across Tennessee. DCYMHS contracts with community providers to provide the most complete continuum of care possible for individuals ages 0-30.

The Tennessee Resiliency Project (TRP) was started with the goals of promoting early childhood mental health, increasing access to school-based mental health services, and ensuring enhanced coordination of crisis care. In FY 2024, 243 children were enrolled in the project, and in 363 reassessments, families reported experiencing reduced disruption of family routines due to the child's emotional or behavioral challenges. Additionally, school attendance increased for 244 enrolled children, 496 children were able to receive mental

health services that had not previously received treatment, and 1,169 children/youth were diverted from inpatient hospitalization.

Office of School-Based Services

Since 1985, TDMHSAS has funded school-based services in Tennessee. In the past fiscal year, these services have been enhanced and expanded. In FY 2024, TDMHSAS allocated \$10M to support 141 additional School-Based Behavioral Health Liaisons (SBBHL) to bring the program total 387 positions statewide, as well as adding seventeen (17) Operational Supervisors and one (1) program evaluator. The Bipartisan Safer Communities Act (BSCA) supplemental MHBG funds enhanced services and supports to children and youth living with SEDs. Through continued MHBG and BSCA supplemental funding, the grant's targeted populations are receiving services in 56 counties with Project BASIC. In FY 2024, 28,846 (2,389 BASIC+ 26,457 SBBHL) youth received behavioral health services and supports such as screening, individualized classroom consultation, and therapeutic interventions in schools through school-based programming; as well as 15,916 (12,091 teachers + 3,825 school staff) school personnel received trainings.

Additionally, the Erase the Stigma program served 22,317 youth. These students received services and supports put in place to decrease the stigma around mental health issues and increase mental health awareness. Erase The Stigma continues to develop a virtual Train the Trainer curriculum that is on track to be released during FY 2025, which will increase the program's reach and be available to all Tennessee counties.

Office of System of Care

The System of Care Across Tennessee (SOCAT) program provides High-Fidelity Wraparound, an evidence-based Intensive Care Coordination model, available in all 95 counties. The SOCAT program is funded by an interagency agreement with the TDHS using TANF funds. In FY 2024, there were 32 SOCAT teams serving 522 families across the state. Ninety-two percent (92%) of those children, youth, and young adults served by SOCAT remained in-home as a result of SOCAT services and supports.

In FY 2024, the Family Support and Advocacy (FSA) program provided support and advocacy for 9,063 parent/caregivers and youth. FSA staff were able to make an impact in 89 counties reaching 1,263 people through trainings and presentations. Sixty-one (61) trainings were offered across the state utilizing both in-person and virtual formats. As a result of the collaborative partnership with the SOCAT Network, the FSA team was able to provide support and advocacy to 45 SOCAT families through the utilization of services from a Family Support Specialist (FSS).

Office of Infant and Early Childhood

The Regional Intervention Program (RIP) is funded by TDMHSAS and is a parent-implemented, professionally supported program for families with young children experiencing challenging behaviors. In FY 2024, 285

target children and 33 siblings from 247 families were served by RIP, and an estimated 370 parents were provided support and education, with graduating families attaining family-specified treatment objectives.

Additionally, RIP continues to provide trainings as requested to the early childhood community. In FY 2024, 2,599 adults received training in RIP strategies, theories, and programming (a 23.8% increase over FY 2023 and a 1,003% increase since FY 2021).

Office of Youth & Young Adults

The First Episode Psychosis Initiative OnTrackTN program is an evidence-based program that addresses the needs of individuals with early SMI, including psychotic disorders. In FY 2024, the FEPI program provided services to a total of 245 youth and young adults.

The growth in number and reach of certified peer programs working with young adults and families continued in FY 2024. The Certified Young Adult Peer Support Specialist (CYAPSS) program for young people ages 18-30 fully certified 35 peers during the year.

Office of Juvenile Justice

During FY 2024, TDMHSAS, in partnership with the Tennessee Department of Children's services (TDCS), Tennessee Commission on Children and Youth (TCCY), Administrative Office of the Courts (AOC), and juvenile judges and court staff, continued implementation of the Juvenile Justice Reform (JJR) Local Diversion Grants using recurring state funding to impact the area of juvenile justice. Through six community-based providers, the program serves youth who have been referred to juvenile court for a delinquent/unruly charge or who have already been adjudicated delinquent/unruly and are at risk of being placed in TDCS custody. The array of services that can be provided include: care coordination, intensive family therapy, group therapy, peer support, medication management, substance use services, crisis planning, school-based or community-based monitoring, respite services, and/or resource linkage. Referrals are received from juvenile court judges, juvenile court staff, and TDCS. During FY 2024, providers were able to serve 1,275 justice-involved youth and their families, and that number is expected to continue to rise.

The Safe Baby Court (SBC) model serves children zero through thirty-six (0-36) months of age and their families. These courts are administered by the DCS, in consultation with the AOC, and TDMHSAS. These specialized courts are grounded in developmental science, focusing on the unique needs of infants and toddlers with a goal of mitigating the impact of adverse early childhood experiences. Three hundred eighty-one (381) children and two hundred twenty-four (224) families were served in FY 2024. Additionally, eight (8) new courts were added to the existing fourteen (14) courts.

Division of Substance Abuse Services

The Division of Substance Abuse Services (DSAS) continues to work to leverage federal and state resources, charting a course in meeting Tennesseans where they are with their substance use issues. While creating successes during FY 2024, DSAS simultaneously worked toward creating pathways of resiliency, recovery, and independence for individuals affected by substance use, mental health, and co-occurring disorders. Our vision of service expansion and creating capacity is at the forefront of what we do to serve those in our state who are most in need.

Strengthening Workforce Support Through Provider Rate Increases

In FY 2024, the TDMHSAS substance use provider network continued to face opportunities as they navigated rising operational costs, increased demand for services, and the need for enhanced staffing and resources. To address these areas and ensure the continued delivery of high-quality prevention, treatment, and recovery services, Governor Lee and the TNGA appropriated \$10.2 million for provider rate increases. By aligning funding with the realities of the current treatment landscape, the rate increases aimed to strengthen the capacity of providers to support recovery and reduce the overall impact of SUDs on individuals and communities.

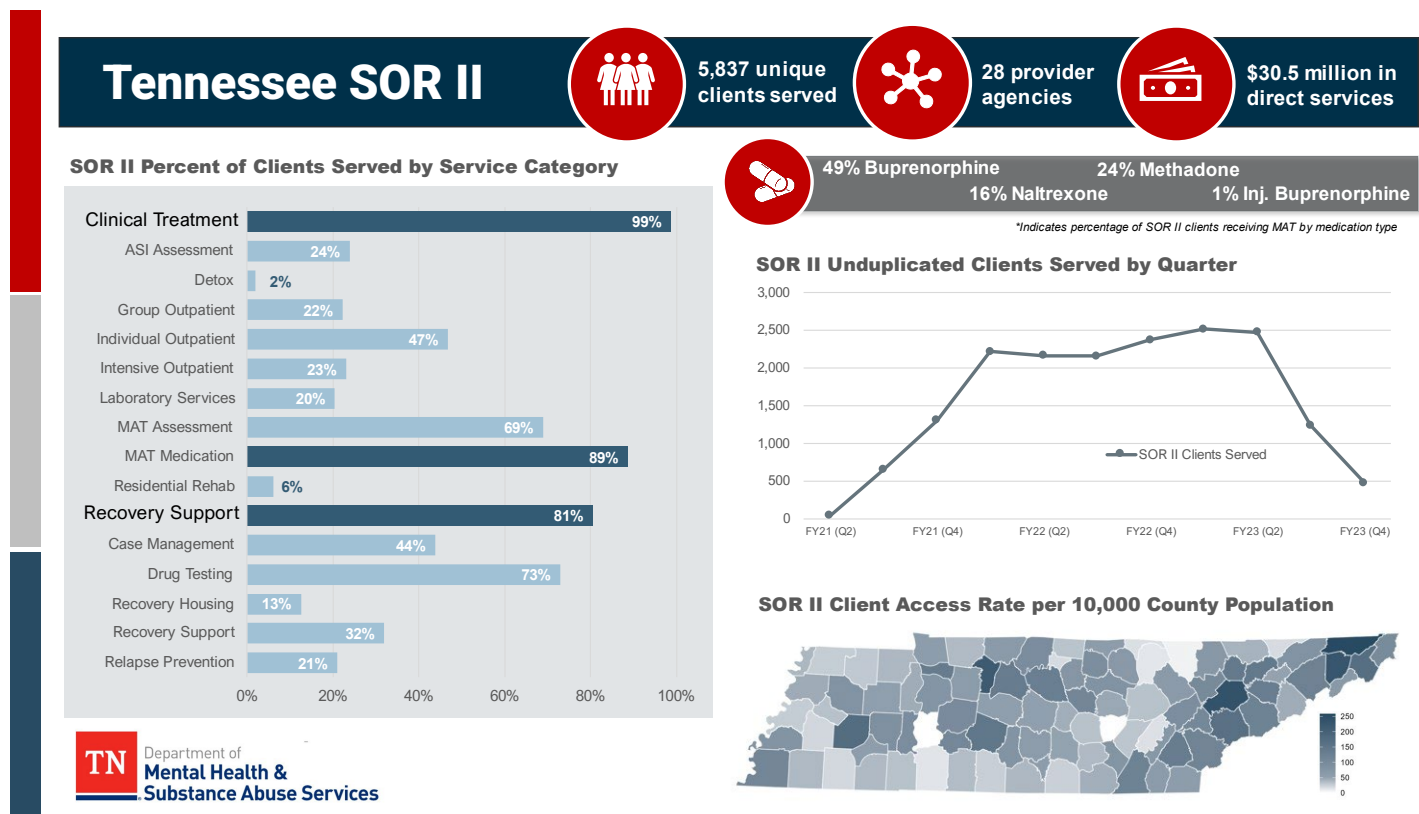
The prevention investment was directed toward recruiting and retaining skilled professionals, improving training and professional development opportunities, and providing necessary support to ensure a sustainable and proficient workforce. These enhancements aimed to not only address immediate staffing needs but also to build long-term capacity for delivering effective prevention and intervention services. By prioritizing workforce development, these additional funds empowered providers and improved program delivery, ultimately reducing the prevalence of substance use disorders and promoting healthier outcomes for individuals and families statewide.

State Opioid Response (SOR) Grant II

The SOR II grant aims to address the opioid crisis by increasing access to MAT using methadone, buprenorphine, and naltrexone, reducing unmet treatment need, and reducing opioid overdose related deaths through the provision of prevention, treatment and recovery activities for opioid use disorder and stimulant use disorder.

SOR II was built on the foundation established by the SOR I grant which included the Hub and Spoke model. This model consists of a network of community providers around a central hub that offers a Medications for Opioid Use Disorder (MOUD) component to individuals seeking services for OUD. There are four (4) regional Hubs within TN located in Johnson City, Nashville, Knoxville, and Memphis (which was expanded to include Jackson). Each Hub provides at least one of the three Federal Drug Administration (FDA)-approved MOUD medications including the addition of Sublocade (injectable formulation of buprenorphine) for OUD and

ensures the medication is available for administration to clients on-site. Currently, there are 24 local Spokes that offer OUD treatment and recovery support services and make referrals to behavioral and primary healthcare.



Prevention and Harm Reduction Outcomes

The Prevention and Harm Reduction initiative focuses on several critical outcomes aimed at reducing opioid and stimulant misuse across the state. The primary goal is to raise awareness of the dangers associated with opioids and stimulants, as well as to promote available resources for overdose prevention. To achieve this, each of the thirteen (13) Regional Overdose Prevention Specialist (ROPS) regions developed and implemented comprehensive media campaigns, reaching diverse populations with critical harm reduction strategies.

ROPS partnered with over 1,900 community agencies, including behavioral health providers, schools, and faith-based organizations, to deliver targeted overdose prevention training. A significant achievement was the increase in naloxone distribution from prior grants, which resulted in the dissemination of more than 325,000 overdose resource kits across the state. These kits were distributed to individuals at high risk of overdose, first responders, and community agencies.

Substance Use Residential Treatment Facilities Infrastructure Grant

In FY 2024, TDMHSAS received \$12 million from the State Opioid Abatement Trust Fund to increase the substance use residential treatment bed capacity in Middle, East, and Southeast areas of the state for the purpose of providing substance use residential treatment services to adult, indigent Tennesseans from all counties. After a competitive request for proposals, Buffalo Valley, Inc. (BVI), the Council for Alcohol and Drug Abuse Services (CADAS), and Helen Ross McNabb were awarded grants which will result in the creation of 156 treatment beds. Projects are anticipated to be completed in FY 2025.

Expansion of Accountability Courts - Mental Health Courts

The TNGA enacted the Mental Health Treatment Act of 2022 to reduce incarceration of justice-involved individuals living with mental illness by diverting them into judicially supervised, community-based treatment programs and services while promoting public safety. This law included the appropriation of funds to implement new mental health court programs and support the continuation of existing mental health court programs. Prior to the inception of the Mental Health Treatment Act, there were nine (9) mental health courts. During FY 2024, eight (8) mental health courts were established for a total of seventeen (17) across the state. The goal is for each Judicial District to have access to a mental health court. DSAS team members continue to organize and participate in informational and planning meetings with judges across the state related to mental health court planning and implementation.

Northeast Tennessee Residential Recovery Court (NTRRC) Programs

In FY 2022, DSAS issued a Request for Proposals (RFPs) to expand its Residential Recovery Court (RRC) Program. RRCs provide services to non-violent, justice-involved individuals who are actively participating in a local recovery court, have a greater need for intensive SUD and/or COD services and supervision, and have not been successful in recovery court programming in their community. The 1st, 2nd, and 3rd Tennessee Judicial Districts came together with the regional vision of decreasing incarceration rates of individuals with SUDs and CODs, and decreasing the negative impact of substances on individuals, families, and communities. The NTRRC concept was awarded grant funds to support this regional effort and it is comprised of both men's and women's programs. In FY 2024, the programs began accepting participants from Carter, Johnson, Unicoi, Washington, Sullivan, Greene, Hamblen, Hancock, and Hawkins counties for a 12- to 18-month program. The men's program operates in the Northeast Correctional Complex and can serve up to eighty-five (85) non-violent male felony offenders. The women's program is in Johnson City and can serve up to ten (10) non-violent female felony offenders.

TN Save a Life Program

Tennessee's Regional Overdose Prevention Specialists (ROPS) continued their lifesaving work in FY 2024. ROPS are located statewide to provide training and education on drug overdose, overdose prevention, emerging substances, and available resources. They also serve as a point of contact for naloxone, fentanyl test strips

(FTS), and xylazine test strips (XTS) distribution in their catchment areas. In FY 2024, ROPS distributed more than 213,000 units of naloxone, an increase of 20% compared to the previous fiscal year (more than 177,000 units). Since the program's inception, TDMHSAS has documented at least 85,000 reversals due to naloxone distributed through the ROPS. In addition, ROPS partnered with more than 4,000 agencies/organizations to provide training, outreach, and resources (naloxone, FTS/XTS, and recovery resources). Through the TN Save a Life Program, formal trainings offered through ROPS were provided to the Department of Health, local health departments, Department of Children's Services, Department of Correction, Rural Health Association, Tennessee Bureau of Investigation, local law enforcement, first responders, K-12 schools, higher education institutions, syringe services providers, substance use treatment and recovery centers, criminal justice providers, mental health and medical providers, civic organizations, and social service agencies. Informal trainings and outreach were offered in a variety of settings including health fairs, resource events, libraries, community centers, local businesses, hotels/motels, encampments of unhoused individuals, jails, and entertainment venues.

Fentanyl Test Strip Education and Distribution

In March 2022, the TNGA passed, and Governor Lee signed legislation that allowed for the individual use of equipment that detects synthetic opioids, such as fentanyl. The most common of these tools are fentanyl test strips (FTS). This low-cost, life-saving tool is now included in the overdose prevention program to support existing efforts to distribute naloxone to people at risk of overdose, and the results have been tremendous. During the first two years of this effort, ROPS and community partners distributed more than 305,000 FTS.

In addition to FTS, xylazine test strips (XTS) are now distributed through community partners and ROPS. Xylazine, an animal tranquilizer increasingly found in illicit drug supplies, presents new challenges due to its resistance to naloxone. Including XTS in the overdose resource kits aims to address this emerging threat. In FY 2024, more than 80,000 XTS were distributed, providing those at risk with more comprehensive detection options to enhance overdose prevention efforts.

Prescription Drug Take Back Days

Take Back Day is a national initiative aimed at reducing access to unused or expired prescription medications, particularly those that can lead to misuse or addiction. Coordinated by the Drug Enforcement Agency (DEA) and supported by state and local partners, these events provide a safe, anonymous way for individuals to dispose of medications that are no longer needed. In Tennessee, TDMHSAS coalitions and community partners have been instrumental in hosting more than 200 events across the state in both October and April annually. These events, held at accessible locations such as hospitals, schools, law enforcement offices, and public parks, offered drive-through and walk-up options, ensuring residents could easily drop off prescription drugs. As a critical prevention strategy, Take Back Day helps limit the availability of medications that could be misused or diverted, reducing the risk of prescription drug misuse and accidental overdose. In FY 2024, more

than 13,000 pounds of prescription drugs were collected in Tennessee, contributing significantly to the broader effort of reducing opioid misuse and protecting public health across the state.

Division of Hospital Services

In FY 2024, the Division of Hospital Services (DHS) continued efforts toward the CFG goal for efficient and effective operation of the RMHIs. There are four (4) RMHIs located across the state of Tennessee:

RMHI	LOCATION	BEDS	FY 2024 ADMISSIONS
Memphis Mental Health Institute (MMHI)	951 Court Avenue Memphis, TN 38013 (901) 577-1800	55	865
Middle Tennessee Mental Health Institute (MTMHI)	221 Stewarts Ferry Pike Nashville, TN 37214 (615) 902-7400	207	2,416
Moccasin Bend Mental Health Institute (MBMHI)	100 Moccasin Bend Road Chattanooga, TN 37405 (423) 256-2271	165	2,578
Western Mental Health Institute (WMHI)	11100 Old Highway 64 West Bolivar, TN 38008 (731) 228-2000	150	536

TDMHSAS continues to partner with three (3) private psychiatric inpatient hospitals in East Tennessee to provide services to the uninsured population in Regions I and II. These hospitals (Parkwest, Ballard Health, and Ridgeview Psychiatric Hospital and Outpatient Center, Inc.) provided 1,719 units of service during FY 2024.

In FY 2024, DHS continued to enhance the services provided to the citizens of Tennessee by utilizing the Patient Bed Matching (PBM) application at all four RMHIs which has produced more efficient admission processes. The PBM application is a web-based tool provided by TDH and is available to assist EDs, crisis providers, and admissions staff in locating and securing an appropriate psychiatric bed at an inpatient behavioral health hospital. The application returns results regarding bed availability to the bed searcher by relying on bed capacity information provided through TDH's Healthcare Resource Tracking System, and provides a secure, Health Insurance Portability and Accountability Act (HIPAA) compliant platform that can be used to share clinical documentation and referral status updates.

During FY 2024, the Department continued to advance two (2) important infrastructure enhancements. The most notable is the construction of a new facility for MBMHI which will begin as soon as a new location site has been identified and secured for building the new facility. Additionally, WMHI has secured a contractor and begun pre-meetings that will lead to Phase 2 construction of a new kitchen and the addition of 25 beds.

The commitment to ensuring quality of services and regulatory compliance across all RMHIs continued this fiscal year. In December 2023, MBMHI underwent an unannounced triennial survey conducted by The Joint Commission (TJC). This TJC survey included a validation survey by the Centers for Medicare & Medicaid Services (CMS) to ensure TJC was holding hospitals to the level of accountability CMS requires. After successful completion of the survey, MBMHI was awarded recertification for another three (3) years.

One of the greatest opportunities experienced by the RMHIs relates to high vacancy rates for crucial positions such as psychiatrists, nurses, custodial workers, and psychiatric technicians. Statewide salary increases have made compensation competitive with the market, significantly improving retention across the system. Additionally, substantial advances were made during FY 2024 after receiving approval to increase hiring rates and salary ranges for some of those key positions and, as a result, there has been a significant reduction in vacancies. Efforts to recruit employees continue including ongoing job fairs, online advertising, accepting walk-in applicants, rapid hiring events, partnerships with nursing and medical schools, a referral bonus program, and the Star Loan Repayment Program (SLRP).

Office of Forensic and Juvenile Court Services

The Office of Forensic and Juvenile Court Services (OFJCS) administers the system for court-ordered evaluations to determine competency to stand trial and mental capacity at the time of the offense, juvenile court-ordered evaluations, treatment for adults to establish competency, commitment for individuals found not guilty by reason of insanity (NGRI), psychiatric evaluations for the Board of Parole, and Mandatory Outpatient Treatment (MOT). In FY 2024, there were 2,579 initial outpatient evaluations which diverted 65% of individuals from the need for an inpatient evaluation. There were 746 inpatient evaluations and 130 new commitments for inpatient treatment of incompetent defendants. Additionally, several new legislative changes were passed during FY 2024, such as Jillian's Law, and the cost for misdemeanor evaluations shifted from the counties to the State, both of which have the potential to effect hospital operations. DHS is monitoring the impact of these changes and is planning for resource allocation as necessary.

In FY 2024, there were 37 evaluations of defendants found NGRI and twenty (20) new NGRI admissions to the RMHIs, numbers which are consistent with pre-pandemic years. Additionally, there were five-hundred forty (540) evaluations conducted by order of juvenile courts and 75 psychiatric evaluations of parole-eligible inmates conducted for the Board of Parole.

The MOT coordinator maintained a database of existing client status throughout the year, tracking 268 active MOT cases receiving services from twenty-seven (27) separate community agencies. The coordinator notified each provider of any MOT cases due for review each month and tracked all cases involving notification of non-compliance to the court. As in previous years, the MOT manual was distributed to providers during trainings conducted in the field and posted on the TDMHSAS website.

More than 27,000 juvenile court screenings have been conducted in the Tennessee Integrated Court Screening and Referral Project since the beginning of the Project in October 2010, facilitating referrals for mental health, substance use, and/or family services. New screener certifications and re-certifications are conducted on an ongoing basis and by the end of FY 2024, over seven hundred (700) juvenile court Youth Service Officers (YSOs) had been trained to complete risk and need screening with youth in juvenile courts.

Office of Strategic Initiatives

Tennessee Recovery Navigators

The Tennessee Recovery Navigators are individuals in long-term recovery who meet patients in hospital emergency departments who have recently overdosed, are in active withdrawal, or are identified as potentially having a SUD and connect them with substance abuse treatment and recovery resources. Since implementation in 2018, the program has continued to grow and to serve more individuals each fiscal year. In FY 2024, the TRNs served 4,561 individuals in EDs across the state, an increase of 831 individuals from the previous fiscal year. This increase can be attributed to the expanded use of telehealth services, increased utilization of in-person Navigation services, and the partnership with the TDH which has afforded the opportunity to expand the number of navigators in specific regions of the state. Of the 4,561 individuals served, 68% were connected to some type of treatment or community resource. Additionally, the program began serving three (3) additional hospitals bringing the total number of hospitals being served to 59.

Project Rural Recovery (PRR)

In FY 2020, TDMHSAS received \$10,000,000 from SAMHSA's Promoting Integration of Primary and Behavioral Health Care grant to implement PRR using two (2) mobile health clinics. In FY 2023, Governor Lee and the TNGA provided ARPA dollars to expand PRR's footprint to include two (2) additional mobile health units. The mobile health clinics provide integrated primary and behavioral healthcare to adults and children in their respective communities; improving access to coordinated, comprehensive care.

Buffalo Valley, Inc. and McNabb Center operate the two (2) original mobile health clinics in ten (10) counties, four (4) of which are rurally distressed (Perry, Wayne, Lewis, Lawrence, Marshall, Cocke, Hancock, Claiborne, Jefferson, and Grainger). Ridgeview Behavioral Health Services and Pathways Behavioral Health began providing integrative mobile healthcare funded through the ARPA funds in FY 2023. Together they add an

additional ten (10) counties (Fentress, Scott, Campbell, Morgan, Union, Lauderdale, Crockett, Haywood, Fayette, and Hardeman) to the already ten (10) being served.



Cumulatively through August 2024, 13,330 visits were provided to 5,331 clients. Data is collected from clients who consent when they begin receiving services and at six-month intervals. This data provides valuable information about improvements clients may experience because of PRR services. During FY 2024, 203 clients reported reduced tobacco use and 45 had lowered blood pressure. Eighteen (18) clients who used the ED for physical health concerns within the last month at baseline did not use the ED within the last month at follow up. Similarly, eight (8) patients who reported using the ED for mental health concerns at baseline did not use it within the last month at follow up.

During Year Three, five (5) questions were added to the assessment focusing on previous care, care alternatives, and travel time. Of the clients who completed the assessments during FY 2024, 800 reported they had not been to a primary care provider in over five years, 794 stated they would have gone to the ED if the mobile clinic were not available, and 466 reported it would have taken over 30 minutes to travel to alternate care.

PRR has successfully helped more than 5,000 rural Tennesseans obtain care they otherwise may not have received ensuring all Tennesseans have access to mental health, substance use, and physical health care.

Workforce Strategic Initiatives

During the special session on public safety conducted in August 2023, the TNGA appropriated \$3,000,000 for the purpose of providing a public behavioral health scholarship program. This program, called the Tennessee Behavioral Health Pathways (TBHP) Scholarship, provides tuition stipends to Tennessee residents pursuing a graduate degree in a behavioral health related field with the requirement that any scholarship recipient work for a TDMHSAS contracted treatment agency for an amount of time equivalent to the amount of time the recipient received scholarship stipends.

These special session appropriated funds are to be expended by June 30, 2025. TDMHSAS anticipates being able to serve 169 students with the scholarship program under the current budget of \$3,000,000. While this is significant, it should be noted that over 800 applications were made for this scholarship before we closed the portal due to the overwhelming response, leaving both pent-up demand and a great opportunity to bring more students into the public behavioral health workforce.

Also during the special session, the TNGA appropriated \$12,131,000 to the TDMHSAS to support sign-on and retention bonuses to prospective or current behavioral health professionals who work for a TDMHSAS contracted treatment agency with the commitment from the employee to stay with the agency for at least one year after receiving the bonus. In FY 2024, \$7,468,511.40 was spent by contracted agencies to provide bonuses to 3,102 employees statewide.

Office of Faith-Based Initiatives

The goals of the Office of Faith-Based Initiatives (OFBI) are to connect individuals struggling with behavioral health challenges to treatment; facilitate understanding of treatment and recovery; increase knowledge of mental health challenges and substance misuse; spread awareness of Faith-Based initiative certification and its requirements; help others understand the continuum of care and collaborate with it; help groups understand and implement the best practice model; and promote and improve the effectiveness of the Faith-Based initiative in bridging the gap between the faith community and recovery support services.

The Lifeline Peer Project was established to reduce stigma related to behavioral health challenges and increase access to recovery support services. Lifeline Coordinators help establish recovery support programs including but not limited to Alcoholics Anonymous (AA), Narcotics Anonymous (NA), Celebrate Recovery (CR), All Recovery, and SMART Recovery, as well as other self-help support groups throughout the state. They also help connect individuals with treatment and speak publicly about their own lived experience in their recovery journey. More recently, the Lifeline Peer Project has incorporated mental health into their training curriculum. There are currently ten (10) Lifeline coordinators located throughout Tennessee.

The Hybrid Lifeline Peer Project was established, in partnership with the TDHS, to serve specific distressed and at-risk regions of the state to increase awareness, coordinate between community resources, and connect individuals with treatment and recovery services. Hybrid Lifeline supports low-income families in receipt of or eligible for either TANF or Supplemental Nutrition Assistance Program (SNAP) and will be eligible to receive support for substance abuse treatment and/or recovery services. There are currently ten (10) Hybrid Lifeline coordinators located throughout Tennessee.

The Collegiate Recovery Initiative was established to inform campuses across the state on how they may take a proactive role in increasing awareness around mental health, curbing the addiction epidemic; equip

campuses across the state with the necessary means to take a proactive role in the mental health of their students and the addiction epidemic across the state; encourage campuses and student bodies to leverage available resources; to assist campuses in the process of assessing for a collegiate recovery college (CRC) or collegiate recovery programs (CRP) and getting their programs off the ground; and to reduce stigma through peer sharing and roundtable discussion (Project Lifeline).

The TDMHSAS Faith-Based Community Coordinators are individuals with lived experience who connect with communities of faith to recruit, train, and certify through the Tennessee Certified Faith-Based Recovery Partner program. Each community coordinator is employed by their local Community Prevention Coalition. Currently, the Department has seven (7) Faith-Based Community Coordinators, one (1) in the northern and southern portions of each grand division of the state, and one (1) specific coordinator for Shelby County.

FY 2024 accomplishments for the Office of Faith-Based Initiatives include:

- One hundred (100) new Certified Recovery Congregations established: YTD 1,151 congregations throughout the state.
- Referred 6,162 individuals into treatment and/or recovery support services.
- Hosted 2,868 events with trainings dealing with stigma, access to recovery support, access to treatment, and understanding mental health. This includes trainings offered via in-person meetings, conference calls, Facebook Live, Zoom, and Webex.
- Around 910 new individuals attended presentations to become Collegiate Recovery Allies.

During FY 2024, surveys were added to the various trainings being provided in the communities and the surveys show:

- 93% of respondents reported the events/trainings were beneficial in helping change their understanding of individuals with behavioral health challenges.
- 96% of respondents report being positively impacted by the presentations provided by our coordinators.
- 89% of respondents report a reduction in personal stigma because of the events/trainings provided by the OFBI.
- 95% of respondents report being more willing to assist someone in finding help with behavioral health challenges because of our events/trainings.

Division of Administrative and Regulatory Services

The Division of Administrative and Regulatory Services (DARS) encompasses the Offices of Licensure, Subrecipient Monitoring, Research, and General Services. The DARS oversees a wide array of TDMHSAS'

critical regulatory and business operations. It also coordinates Information Technology (IT) and fiscal services with the Division of IT, Division of Fiscal Services, and the TDMHSAS Budget Office.

The Office of Licensure currently licenses 1,921 personal support, mental health, and substance abuse treatment sites across Tennessee. These licensed sites are operated by 1,101 unique organizations. In FY 2024, the Office of Licensure performed 2,340 licensing oversight visits. An “oversight visit” is an on-site inspection or investigation or a virtual off-site inspection. Additionally, the office fielded 4,283 complaints, which resulted in 3,197 incident investigations. Finally, Licensure administratively facilitated the placement of 19 individuals on the State of Tennessee Vulnerable Adult Abuse Registry for their abuse, neglect, and/or misappropriation of a vulnerable person in FY 2024.

The Office of Subrecipient Monitoring (OSM) develops the Department’s annual Subrecipient Monitoring Plan for submission to the State’s Central Procurement Office. In FY 2024, the OSM fiscal monitors reviewed more than 352 contracts within 92 agencies. OSM also continues to provide one-on-one technical assistance to subrecipient agencies. In FY 2024, the OSM provided technical assistance to 100 agencies and monitors the cognizant agency list to determine if grantees require audits and/or single audits. Currently, OSM monitors 130 cognizant agencies of which 48 required audits and reviews the majority of new and revised Cost Allocation Plans and Indirect Cost Rates for all cognizant agencies in coordination with the Department of Finance and Administration’s (F&A) Fiscal Services. OSM remains the Department’s liaison with the Tennessee Comptroller of the Treasury and is charged with conducting fraud, waste, and abuse investigations on behalf of the Department. This year, OSM received four (4) internal investigations as well as 20 investigations from the Comptroller’s Fraud, Waste, and Abuse Hot Line, 17 of which were investigated by OSM, with the remainder investigated by other Department divisions.

Division of General Counsel

The Division of General Counsel supports the Department’s operational and administrative activities by providing Department-wide legal services, investigating internal complaints, processing contracts, and serving as privacy, ethics, and compliance counsel. In the past fiscal year, the Office of Legal Services engaged in more than 5,500 court actions (primarily probable cause and judicial commitment hearings related to state psychiatric hospital admissions), handled numerous abuse registry cases, disciplinary appeals actions, Equal Employment Opportunity Commission (EEOC) and Tennessee Human Rights Commission (THRC) complaints, and other miscellaneous legal actions and projects. Additionally, the Office of Investigations (OI) conducted over one hundred thirty-two (132) investigations and the Office of Grant and Procurement Contracts processed 1,940 contracts and amendments worth more than \$504 million.

Division of Clinical Leadership

The Division of Clinical Leadership (DCL) is responsible for providing clinical oversight and policy development for the RMHIs and clinical consultation to various divisions and offices within the department. The DCL oversees Tennessee's Opioid Treatment Programs (OTPs); coordinates training and support for suicide prevention initiatives in the African American faith communities; ensures Title VI compliance for the department; and provides training in collaboration with the DSAS. The DCL also pilots substance abuse initiatives designed to benefit individuals and/or families, and partners with community and other state agencies to provide training and educational opportunities on a variety of mental health and substance abuse topics.

In FY 2024, the DCL was responsible for two grants that addressed the complex needs of families with children at risk of out-of-home placement due to parental substance misuse: the Therapeutic Intervention, Education and Skills (TIES) grant and the TDMHSAS-HOMEBUILDERS (TDMHSAS-HB) grant. The TIES grant was funded by TDHS and the TDMHSAS-HB grant is funded through the Federal Administration for Children and Families (ACF). Both grants targeted eligible families in nine (9) rural counties in the South Central DCS Region: Bedford, Coffee, Franklin, Giles, Hickman, Lawrence, Lincoln, Marshall, and Maury. While children in TIES families were at risk of removal from the home, the children in TDMHSAS-HB families were at imminent risk of removal.

In FY 2024, TIES provided services to 36 unduplicated families involving a total of 123 individuals (67 children and 56 adults). One hundred percent (100%) of TIES focal children were safely and successfully maintained in their homes at case closure. As of June 30, 2024, funds ended for TIES, and there are no longer TIES services in the designated counties in the South Central DCS Region. TDMHSAS-HB served twelve (12) families (25 children and 19 adults) with zero removals following services.

Additionally, the DCL collaborates with TDH to identify mental healthcare professional shortage areas (MHPSAs), working diligently to ensure that these designations remain up to date. Shortage area designations tend to enhance recruitment and retention due to the availability of loan repayment programs (LRPs) for behavioral health practitioners that agree to provide services in those areas. In January 2024, identified census tracts in Murfreesboro became MHPSAs. The DCL informs contracted providers on the process to become a National Health Service Corps- (NHSC-) approved site annually. Employees and potential employees in specified disciplines that work at NHSC-approved sites can then apply to loan repayment programs. In FY 2024, the Tennessee State Loan Repayment Program (TSLRP), administered through TDH, awarded \$130,000 in loan repayment funding to eligible behavioral health practitioners. There were three (3) new awardees, one (1) full-time and one (1) part-time licensed marriage and family therapist (LMFT), and one (1) psychiatric mental health nurse practitioner (PMHNP). The additional awardees were continuations and granted to a full-time PMHNP and a part-time licensed clinical social worker (LCSW). Further, four (4) practitioners at MTMHI

received loan repayment awards through the Substance Use Treatment and Recovery (STAR) LRP in FY 2024: one (1) psychiatrist, one (1) psychologist, one (1) NP, and one (1) RN.

In FY 2024, the Nashville Suicide Prevention in the African American Faith Communities Coalition (NSPAAFC) continued to focus on distributing mental health and suicide prevention resource materials to additional faith communities and local organizations in the Nashville area, hosted an increased number of mental health and suicide prevention exhibits at community events, provided training, and added new members to the coalition. The coalition planned, coordinated, and facilitated SPAAFCC's 8th Biennial Suicide Prevention Conference on August 8, 2024. Over 164 people participated in this conference.

In addition to developing and submitting the department's annual Title VI Compliance Report and Implementation plan for FY 2024, the DCL, in collaboration with DSAS staff, coordinated and trained over 450 contract agencies' Title VI coordinators and their other staff during the Title VI training via WebEx in May and June of 2024. DCL staff also provided technical assistance to an increased number of contract agencies' new Title VI coordinators and processed several Title VI complaints in 2024.

The Chief Medical Officer (CMO) continues to assume increased responsibility for oversight of psychiatric health care delivery at the three (3) private psychiatric contract hospitals. To better monitor and manage patient LOS at those facilities, opportunities for improvement in treatment are offered at the time of request for increased LOS and rationale for increased stay has been critically reviewed on a near-daily basis. A process for consultation between the three (3) contract hospitals and their referral RMHI has been developed which has encouraged communication among doctors and enhanced patient care.

The CMO and other staff within the DCL continue to partner with the Bureau of TennCare, TDOC, and multiple universities within the Nashville area on training and workforce development.

Division of Planning, Policy & Legislation

The Division of Planning, Policy & Legislation (DPPL) is comprised of two (2) offices: the Office of Planning and the Office of Legislation and Rules.

The Office of Planning produces the Department's Three-Year Plan, administers the Statewide and Regional Planning and Policy Council system, coordinates appointments to the Planning Councils, and develops and submits the MHBG application, the MHBG Annual Report, and the Department's Joint Annual Report (JAR). In FY 2024, the Office of Planning continued to empower the Councils to fulfill their roles as required by law, become more active and effective by increasing membership, specifically among service recipients and their family members, increase mental health and substance abuse awareness activities within the community, and encouraging more purpose-driven meetings. Regional Councils were also encouraged to remain involved in

the Legislative Proposal process and, as a result, submitted a total of three (3) legislative proposals for review. In addition, Office of Planning staff engaged in special projects including providing technical assistance to Statewide and Regional Councils, providing trainings to new Statewide Council members, consulting with other Divisions and Offices, and developing relationships with other state and federal agencies. In collaboration with the Office of Research, the Office of Planning continued the ongoing process of increasing the use of data to inform decision-making and ensure a data-driven Needs Assessment process through the updated Fast Facts Portal, which is available for the Councils to use in the Needs Assessment process.

The Office of Legislation and Rules tracked 533 bills during the 2024 legislative session and produced an [annual legislative summary](#) of legislation that became law in 2024 for review and use by TDMHSAS Executive Staff and the Statewide and Regional Planning and Policy Councils. Two (2) legislative proposals from TDMHSAS Executive Staff were submitted for review and consideration and both of those proposals, regarding removing obsolete references to the federal Drug Addiction Treatment Act (DATA) Waiver that pertains to the prescription of certain controlled substances and allowing a Qualified Advanced Practice Provider to sign one of the Certificates of Need for the probable cause and judicial commitment hearings as long as the Qualified Advanced Practice Provider is not in a collaborating agreement with the licensed physician who signed the other Certificate of Need, were included in Governor Lee's 2024 Legislative Package and became law in the Summer of 2023. See Tennessee Public Chapter 575 of 2024 and Tennessee Public Chapter 785 of 2024. Additionally, the Office gave counsel to the Department regarding proposed, potential, and pending rulemaking activity. Specifically, the Office worked in conjunction with the DCL and other state agencies to coordinate the promulgation of rules related to scheduling of controlled substances as required by state law. These rules became effective on May 30, 2024. In addition, the Office collaborated with TDMHSAS Executive Staff and Divisions to spearhead a new statutorily required project set forth in T.C.A. 4-5-213, requiring all state departments with rules to review their department rules and create a report for the General Assembly due by December 1, 2023, and every eight (8) years thereafter.

Office of Communications

State fiscal year 2024 showcased a continued emphasis on storytelling for the TDMHSAS Office of Communications (OC) with special emphasis around the department's key initiatives. The OC published 12 news releases, 10 newsletters, and 890 social media posts. Social media efforts were marked by continued challenges navigating algorithms to achieve organic reach as platforms continue to monetize the ability to reach audiences that were once attainable. The following chart shows key stats and year over year growth from FY 2023 to FY 2024.

	Total Followers	Year Over Year Increase	Impressions
Facebook	16,543	1,002	2,431,848
Instagram	1,833	166	638,745
LinkedIn	5,485	2,043	205,929

Key initiatives and areas of focus for the Office of Communications in state fiscal year 2024 included:

Continued Awareness of 988 Suicide and Crisis Lifeline

The OC supported efforts to continue to spread awareness of the 988 Suicide and Crisis Lifeline in Tennessee with paid marketing campaigns and earned media opportunities. Paid campaigns included the debut of four new, locally-produced commercials focused on parents and young adults ([Link to YouTube Playlist](#)). These commercials plus the existing, Emmy Award nominated spot featuring crisis calltakers combined for nearly 7 million impressions in a limited run in the spring of 2024. The Office was also integral in educating the public through earned media opportunities due to large interest from media outlets across the state. The OC developed talking points, contributed to conference calls, and coordinated interviews for department leaders and community providers.

Employee Recruitment

The OC continued its expanded work in partnership with the Office of Human Resources. The OC provided graphic design, social media marketing, and other support functions as Central Office and RMHI Human Resource leaders continued their efforts to recruit and hire new staff. [YouTube Playlist link to employee growth success story](#).

Substance Use Prevention

The Office continued its close collaboration with the DSAS Office of Prevention and Early Intervention Services to develop materials, promote resources, and generally increase awareness of emerging drug trends. Collaborative efforts around major prevention events including International Overdose Awareness Day and National Prescription Drug Take Back Days proved fruitful with extensive opportunities for earned media generated.

Regional Planning and Policy Councils

In FY 2024, Regional Planning and Policy Councils continued successfully to work towards advocacy of mental health and substance use issues within their respective regions, increasing membership of service recipients and family members to aid in the annual Needs Assessment process, and garnering more active participation on their Adult, Children's, and Legislative subcommittees. These subcommittees are not only vital in the annual needs assessment process, but also in the development of legislative proposals. Two (2) legislative

proposals from the Region IV Planning and Policy Council and one (1) legislative proposal from the Region V Planning and Policy Council were submitted for review and consideration.

Regional Planning and Policy Councils continued to work productively through education, support, and advocacy to meet the Department's vision to be a state of resiliency, recovery, and independence in which Tennesseans living with mental illness and SUDs thrive. While Regional Planning and Policy Councils participated in advising the Department on the Three-Year Plan through the annual needs assessment and legislative proposal process, they also continued their work in advocating for children and adults with mental health and substance abuse needs by participating in mental health and substance use awareness events across the state.

Opportunities

Workforce Development

All sectors of the workforce are experiencing staffing challenges in the post-pandemic employment landscape, and the public behavioral health field is no different. Funding added by the TNGA in August 2023 and in the FY 2024 state budget supported CMHPs as they worked to recruit and retain staff to serve the Tennesseans with the greatest behavioral health needs. Additionally, DSAS developed a comprehensive approach focused on enhancing skills, fostering professional growth, and building long-term sustainability in prevention efforts. This comprehensive approach includes internships to develop future prevention professionals, financial bonuses to encourage achieving and/or maintaining Certified Prevention Specialist (CPS) certification, and support for participation in the National Coalition Academy (NCA) to strengthen coalition skills and leadership. Additionally, the Coalition Business Administration Certificate program will be offered to provide training in financial management and strategic planning to enhance operational capacity. These initiatives are designed to build a skilled, credentialed workforce ready to address substance use challenges effectively. The Department also continues to seek opportunities to address workforce issues at the RMHIs.

Opioid Epidemic

Tennessee continues to experience a significant shift in the opioid epidemic, transitioning from prescription drug use to substances laced with synthetic opioids, most notably illicitly manufactured fentanyl. While overall opioid-related deaths have steadily risen from 1,306 in 2018 to 3,073 in 2022, the rate of increase has slowed slightly in recent years, potentially reflecting the impact of ongoing prevention, treatment, and recovery efforts.

TDMHSAS is addressing this issue by leveraging an increase in state and federal funding to implement strategic and innovative countermeasures including increasing the distribution of medication lockboxes; deactivation pouches (a method used to deactivate unwanted prescription pills and allow them to be safely

discarded); increasing training opportunities on opioids, stimulant, and emerging drugs; increasing education on and access to fentanyl/xylazine test strips and naloxone; community outreach around available resources; increasing utilization of MOUD; expanding recovery support services, and engaging EDs to assist them in their ever important role in combating OUD. Additional opportunities to enhance MOUD options include adding injectable buprenorphine and increasing access to MOUD and clinical treatment.

In addition to these strategies, TDMHSAS-funded partners from across the state actively participate in outreach and education events such as DEA National Drug Take Back Day, which promotes safe disposal of unused medications to prevent misuse, and International Overdose Awareness Day, which raises awareness and honors those lost to overdose while emphasizing the importance of harm reduction. These events, along with other community-driven initiatives, play a vital role in educating the public, reducing stigma, and encouraging safer behaviors. Collectively, these efforts are tailored to address the rise in illicitly manufactured fentanyl and other synthetic opioids, while supporting individuals and communities on their path to recovery. TDMHSAS continues to provide administrative support to the Opioid Abatement Council (OAC).

Stimulant Use Disorder

Deaths involving both opioids and stimulants have continued to rise significantly over the past five years. TDMHSAS will address the risk factors of OUD, opioid overdose, and stimulant use disorder by utilizing TDMHSAS-funded SOR providers for opportunities to provide treatment services in addition to Federal Block Grant funding for the continuum of care. Risk factors will also be addressed by including individual-based prevention providers, substance use prevention coalitions (SUPCs) and ROPs.

Reducing Stigma

Reducing the stigma associated with mental illness or substance abuse disorders will allow individuals to feel more empowered to seek treatment. TDMHSAS will continue investing in education, prevention, and awareness efforts to reduce stigma of mental illness and substance abuse disorders.

Suicide Prevention

According to the most recent data available from the Office of Vital Statistics, 1,245 Tennesseans lost their lives to suicide in 2022. While TDMHSAS, Centerstone, the Jason Foundation, and the Tennessee Suicide Prevention Network (TSPN), among others, strive to prevent suicide through training, awareness, counseling, and outreach, reducing the number of suicides remains a challenge throughout the state. TDMHSAS and the TSPN will continue to aggressively address this issue.

Council-Identified Challenges

In FY 2024, Regional Planning and Policy Councils identified several needs across the state. The 2024 Needs Assessment Summary can be found at this link: <https://www.tn.gov/behavioral-health/planning1/needs-assessment.html>

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